

--

Name: خيريه سيد عبد السميع عبد السلام National ID: 27107242200781	
--	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Bilateral mammogram and breast ultrasound
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



٢٠٢٢/١٠ ٢٧١.٧٢٤٢٢.٠٧٨١

فلاحه

ارملة

مسلمة

انثى



البطاقة سارية حتى ٢٠٢٩/١٠/١٤



جمهورية فلسطين العربية
بطاقة تحقيق الشخصية

خيريه

سيد عبدالمصعب عبد السلام

قرية فارون

مركز يوسف الصديق - الخيول



٢٧١.٧٢٤٢٢.٠٧٨١

JQ2520017

EV 13
C.C.I

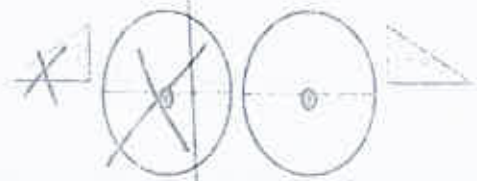
MDT FORM

12/20/2013

1/2/13
C.C.C

Name	gawel us un 2013	MRN	111971101
Age	50 ys	ID	2111111111111111
Menopausal status		Phone	
OCP		marital status	
Co morbidity	pre-diabet	Family history	

Clinical Data

Bx:				R	L
ER: 6/8	PR: 4/8	HER 2: -ve	Ki67: 35%		
Sonomammography:				RT. HRM → 20 days ago. T N M	
Mets Workup:					

Recommendations:

- yp T2 > yp N2
- LNs (4/18)
- Free margins / Free deep margins.
- IDC GII + minimal DCIS Components.
- positive lymphovascular tumor emboli

MDT OUTCOME & PLAN

plan for AS + 2 cycles
+ Adi 2 cycles q 6
+ Adi R/L

SIGNATURE

MEDICAL ONCOLOGY

SURGERY

RADIOLOGY

PATHOLOGY

1/2/13

1/2/13



ImmunohistoChemistry report

Report Number: m5124

Female :	النوع	خيريه سيد عبدالسميع	اسم المريض :
16/10/2021 11:02 :	التاريخ	50 Y	السن :
18/10/2021 12:19 :	تاريخ استلام العينة	مركز اورام الفيوم	تاريخ اخذ العينة :
	تاريخ التقرير	مركز اورام الفيوم	طالب التحليل :
			عناية :

Primary Sample :

Paraffin Block

Test :

ER & PR & HER2 neu & Ki67 (ER&PR: Harvey scoring system -Her2neu : CAP scoring system)

Comment :

Technique :

sample recieved was treated by: - CONFIRM anti-estrogen (ER) (SP1) Rabbit monoclonal primary antibody.- CONFIRM anti-progesterone receptor(PR) (IE2)Rabbit monoclonal primary antibody.

Sample recieved was treated by: -VENTANA anti-HER2/neu (4B5) Rabbit monoclonal primary antibody using DAB Detection kit in ventana benchmark GX.

Sample recieved was treated by: - CONFIRM anti Ki67 (30-9) Rabbit monoclonal primary antibody.

Result :

ER: moderate nuclear staining of 60 % of tumour cells

PR: moderate nuclear staining of 10 % of tumour cells

HER2/neu protein over expression : No membrane staining of tumor cells.

Ki67 :positive nuclear staining of 35 % of tumour cells.

Referral Lab :

Comment :

Conclusion :

ER: POSITIVE (SCORE 6/8)

PR: POSITIVE (SCORE 4/8)

HER2/NEU PROTEIN OVER EXPRESSION : NEGATIVE

#Ki67: THE PROLIFERATION INDEX OF THE TUMOUR IS 35 %

Pathologist:

د/أيمن ومزى

Lab Director: د/ تريزا عاهر

18/10/2021

Patient Name : الأستاذة / خيرية سيد عبدالمنيع عبدالسلام :	Request No. : 13004323
Patient No. : 27107242200781	Request Date : 29/06/2022
Path No. : 1858/22	Verification Date : 18/07/2022
Sex : Female	Financial Category : نفقة الدولة - ورام دار السلام
Age : 50 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report**Clinical Data:**

Right breast cancer; Post NACT

Nature of the specimen:

RT MRM

Gross Examination:

- Procedure: RT MRM specimen measured 25x23x5cm, covering skin measured 21x20cm.
- Tumor extent: An ill-defined grayish white rubbery to firm tumour bed site
- Size: 5.5x4x3cm
- Localization: UIQ
- Distance from margins: 1cm away from the skin, and 2cm from deep margin.
- Lymph nodes number and size: 17 LN, up to 2.5x1cm

Microscopic examination:

- Histological type: Invasive duct carcinoma (70% cellularity).
- Grade (differentiation+ nuclear pleomorphism + mitosis): GII (3+2+1)
- Lymphovascular tumour emboli: Positive
- Perineural invasion: No
- Ductal carcinoma in situ: Minimal DCIS less than 5%
- Lymph node involvement: Positive metastatic deposits in (4/17) LN, with focal extracapsular invasion.
- Resection Margins: Free deep margin
- Free nipple (showed lymphovascular tumour emboli)
- Histologic response to therapy: Partial pathological response
- Additional pathologic findings: No

Diagnosis:

Verified by :

abdelrahman ibrahim

Printing Date : 20/02/2024 12:58:27

Page 1 of 2

Printing By : Mrs.Mayada Hegazy

Patient Name : الأستاذة / خيرية محمد عبد السمیع عبد السلام	Request No. : 13004323
Patient No. : 27107242200781	Request Date : 29/06/2022
Path No. : 1858/22	Verification Date : 18/07/2022
Sex : Female	Financial Category : نوزام دار السلام - نفقة الدولة
Age : 50 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

Right breast cancer; RT MRM:

- Invasive duct carcinoma grade II (70% cellularity), and minimal DCIS components.
- Positive lymphovascular tumour emboli.
- Positive metastatic deposits in (4/17) LN, with focal extracapsular invasion.
- Free deep margin.
- Free nipple (showed lymphovascular tumour emboli).
- Partial pathological response.
- Stage: ypT2, ypN2
- Residual cancer burden is 4.5; CLASS: RCB-III

Prof. Dr. Lubna Omar
Dr. Abdulrahman Ibrahim

Verified by :
abdelrahman ibrahim





مطبوعه

Name : Khayraya Sayed Date : 08/11/2021

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

1. Chest:

- Right breast UOQ irregular speculated mass lesion measuring 6.5 X 6 cm seen invading the skin with diffuse skin thickening.
- Right axillary level I and II pathologically enlarged lymph nodes the largest measures 4.5 x 2.6 cm.
- prominent right internal mammary lymph nodes measuring 10x7mm.
- Few bilateral scattered tiny pulmonary nodules ranging from 2-3 mm.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. Abdomen and pelvis:

- The liver is enlarged with homogenous CT texture. No definite enhancing focal lesions or contour irregularities. Right hepatic lobe segment VIII calcific focus mostly representing post granulomatous sequel.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.



عبدالله

- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- The scanned skeleton shows left anterior acetabular sclerotic focus mostly bone island.

OPINION:

- *Right breast malignant looking mass lesion with metastatic ipsilateral axillary and internal mammary lymph nodes as described.*
- *Bilateral scattered pulmonary nodules likely non-specific for follow up.*
- *No definite hepatic or osseous metastatic deposits.*

Dr. Mohammed Mohsen, MSc.

Dr. M. Goma, M.D.

خيرية عبد السيد عبد السميع

Patient name:

Date: 21/11/2021

Combined bilateral breast mammography and complementary ultrasound

Indication:

Right breast palpable mass.

Technique

- Bilateral MLO and CC views
- Bilateral 3D MLO DBT (Digital Breast tomosynthesis)
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
 - Right retro areolar and UOQ irregular spiculated dense mass with overlying suspicious groups of microcalcifications associated with nipple infiltration and overlying skin edema. Roughly measuring about 14 x10 x13cm.
 - Left benign macrocalcification , no suspicious micro calcifications or architectural distortion detected.

Complementary ultrasound :

- Right breast retro areolar and UOQ irregular heterogenous spiculated mass ,surrounded by echogenic hallow (desmoplastic parenchyma) , seen attached to overlying skin and associated with nipple retraction with highly internal vascularity seen within.
- Left UOQ IMLN.
- No left cystic or solid mass lesions.
- Normal left skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
- Multiple right pathological axillary (level I,II,III) LNs with diffuse cortical thickening and effaced fatty hilum the largest measuring about 50 x27mm.
- No enlarged lymph nodes seen supra-clavicular.
- Nonspecific left axillary LN.

Opinion:

- Right locally advanced malignant mass with ipsilateral pathological axillary level I,II,III , (BIRADS5) for histopathological correlation
- left study benign finding BIRADS 2

Best regards,
Dr\ Nehal hamdey, FEBR, MSC.

Patient Name: Khaireya Sayed Abdel Sameea Abdel Salam	Date : 20/02/2024
Exam : MAMMOGRAPHY BOTH	Age : 52 Year(s)
Medical ID: 4723/2021	Pat. ID: 13529

**DIGITAL SOFT TISSUE MAMMOGRAPHY AND COMPLEMENTARY
ULTRASOUND**

Indication:

Patient is 52 years old and is seen for follow-up after right MRM

Report:

- Right MRM:
 - Operative bed shows no mass or collection
 - No regional lymphadenopathy.
- Left Breast:
 - Scattered fibroglandular parenchymal densities (ACR b)
 - No suspicious micro calcifications or architectural distortion detected.
 - Complementary ultrasound no cystic or solid mass lesions.
 - Normal skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
 - Nonspecific axillary LN.

Opinion:

- Post right MRM status showing no evidence of recurrence (BIRADS 2)
- Normal sonomammography of left breast (BIRADS 1)

Best regards,
Dr. Amira A Radwan MD,
Breast Imaging consultant,
NCI, Cairo University
20/02/2024 12:25 PM



Patient Name: Khirya Saeed Abdel-Salam.

Date: 2/1/2022

^{99m}Tc-^{99m}Tc-MDP BONE SCAN

Desired information : *skeletal surveying to detect multiple lesions.*

Technique: Anterior and posterior whole body scan performed 2 hours after I.V administration of 20 mCi ^{99m}Tc -MDP;

Report:

- Degenerative changes are noticed over dorso-lumbar vertebrae.
- No definite hot spot that account for osseous deposits could be detected all over the surveyed skeleton.
- Both kidneys are seen.

Conclusion:

The surveyed skeleton is free of any active osseous lesions.

Thanks for reference

Nuclear Medicine Unit

DR. Sherin Mahdy, MD

جمهورية مصر العربية
بطاقة تحقيق الشخصية

خيريه
سيد عبدالسميع عبدالسلام
قرية قارون
مركز يوسف الصديق - الفيوم

١٩٧١/٠٧
٢٧١٠٧٢٤٢٢٠٠٧٨١

JQ2520017



s Name:

Khayriya Said Abdel Samie Abdel Salam

2026-06-30

Prof.Dr. no doctor

Goustave Roussey International Egypt

red by:

Hospital:



MULTISLICE CT EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS WITH

CONTRAST ADMINISTRATION REVEALED:-

- Evident right breast mastectomy(MRM) on top of cancer with the current study shows clear operative bed with no evidences of either recurrent /residual neoplastic lesions seen herein.
- Normal left breast.
- Benign -looking left axillary lymph nodes are seen with preserved their hilar fat contents and the largest one measures about 19x5.7mm in dimensions.
- Hyperinflated both lungs.
- Ground-glass opacity is seen in the middle lobe and anterior segment of left lower pulmonary lobe with thin linear atelectatic bands also seen in the former.
- Minimal pleural reaction is seen bilaterally.
- No sizable abnormally-enhancing pulmonary nodules ,patchy areas of consolidations or foci of calcifications noted.
- No sizable abnormally-enhancing mediastinal or hilar lymph nodes seen.
- Patent central airways.
- No pleural or pericardial effusion.
- Border line size of the heart with normal CT of its big vessels.
- Normal bones of the thoracic cage.
- Border line sized liver,measuring about 16 cm, showing a homogenous parenchymal attenuation and enhancement pattern.
- No sizable abnormally-enhancing focal lesions or dilated biliary radicles.
- Patent PV.
- Average sized spleen,measuring about 10.4 cm. No focal lesions seen inside.
- Normal CT appearance of the pancreas, functioning both kidneys
- both suprarenal gland, aorta and IVC.
- No sizable abdominal or pelvic lymphadenopathy.
- No ascites.
- No intra-vesical stones,solid masses,wall thickening or diverticulosis seen.
- Cuts taken in the pelvis are unremarkable.

IMPRESSIONS:

- Evident right breast mastectomy(MRM) on top of cancer with the current study shows clear operative bed with no evidences of either recurrent /residual neoplastic lesions seen herein.

<i>Patient Name: Khaireya Sayed Abdel Sameea Abdel Salam</i>	<i>Date : 11/08/2025</i>
<i>Exam : MRI DORSAL SPINE</i>	<i>Age : 54 Year(s)</i>
<i>Medical ID: 4723/2021</i>	<i>Pat. ID: 13529</i>

MRI OF THE CERVICAL AND DORSAL SPINE

Technique :

Sagittal T1, T2
Axial T1 & FE

Findings:

- Straightened spinal curve with good vertebral alignment
- No vertebral structural collapse or marrow signal alteration.
- Spondylodegenerative changes evidenced by:
 - Marginal anterior osteophytic lipping of the opposing vertebral endplates.
 - Degenerated intervertebral discs evidenced by reduced height & loss of their bright T2 W1 signal.
- C4/5 mild posterior disc bulge seen effacing the ventral subarachnoid CSF space and indenting the cord.
- Normal caliber & MR signal of the cord. No cord oedema, malacia or syrinx.
- No paravertebral masses or collections

OPINION:

- *Spondylodegenerative changes with C4/5 posterior disc lesion as described.*

Dr. Sherief Wagdi

16/08/2025 2:35 PM

Patient Name: Khaireya Sayed Abdel Sameea Abdel Salam	Date : 11/08/2025
Exam : MAMMOGRAPHY BOTH	Age : 54 Year(s)
Medical ID: 4723/2021	

**LEFT BREAST MAMMOGRAPHY WITH BILATERAL
ULTRASOUND**

INDICATION:

- Right MRM

TECHNIQUE:

- Left breast MLO and CC views
- Bilateral ultrasound

REPORT:

- Fairly dense parenchyma, (ACR B).
- No spiculated masses or grouped microcalcifications.
- Complementary U/S also showed no left breast solid or cystic masses.
- A left benign calcification is noted.
- Normal skin thickness and nipple-areola complex.
- Left nonspecific axillary nodes, with preserved fatty hila and uniform cortical thickness.
- Free right operative bed with no evidence of recurrent mass lesions, and free infra and supra clavicular regions.

OPINION:

- Right MRM with free operative bed.
- Left benign calcification, (BIRADS 2).

**Best regards,
Dr. Mahi Mokhtar**

Patient Name: Khaireya Sayed Abdel Sameea Abdel Salam	Date : 11/08/2025
Exam : MAMMOGRAPHY BOTH	Age : 54 Year(s)
Medical ID: 4723/2021	

LEFT BREAST MAMMOGRAPHY WITH BILATERAL ULTRASOUND

INDICATION:

- Right MRM

TECHNIQUE:

- Left breast MLO and CC views
- Bilateral ultrasound

REPORT:

- Fairly dense parenchyma, (ACR B).
- No spiculated masses or grouped microcalcifications.
- Complementary U/S also showed no left breast solid or cystic masses.
- A left benign calcification is noted.
- Normal skin thickness and nipple-areola complex.
- Left nonspecific axillary nodes, with preserved fatty hila and uniform cortical thickness.
- Free right operative bed with no evidence of recurrent mass lesions, and free infra and supra clavicular regions.

OPINION:

- Right MRM with free operative bed.
- Left benign calcification, (BIRADS 2).

**Best regards,
Dr. Mahi Mokhtar**

Patient Name: Khaireya Sayed Abdel Sameea Abdel Salam		Date : 11/08/2025
Exam : MRI DORSAL SPINE		Age : 54 Year(s)
Medical ID: 4723/2021		Pat. ID: 13529

MRI OF THE CERVICAL AND DORSAL SPINE

Technique :

Sagittal T1, T2
Axial T1 & FE

Findings:

- Straightened spinal curve with good vertebral alignment
- No vertebral structural collapse or marrow signal alteration.
- Spondylodegenerative changes evidenced by:
 - Marginal anterior osteophytic lipping of the opposing vertebral endplates.
 - Degenerated intervertebral discs evidenced by reduced height & loss of their bright T2 WI signal.
- C4/5 mild posterior disc bulge seen effacing the ventral subarachnoid CSF space and indenting the cord.
- Normal caliber & MR signal of the cord. No cord oedema, malacia or syrinx.
- No paravertebral masses or collections

OPINION:

- *Spondylodegenerative changes with C4/5 posterior disc lesion as described.*

Dr. Sherief Wagdi

16/08/2025 2:35 PM